

Title: ADHERE: Change in INCAT Leg Score from Stage A Baseline	
Project Owner: Marilla Cherveney	Date: September 22, 2025
Agency Contact(s): N/A	

Purpose: This data on file packet is the reference for the following information: In messaging market research and multiple advisory boards, data on the change in INCAT leg score is continuously ranked by HCPs as the most important efficacy data point. HCPs do not use the INCAT scale in clinical practice, and are therefore hindered in their ability to understand the clinical utility of VYVGART Hytrulo. The change in INCAT leg score data is key to helping them understand how improvement on the INCAT scale translates to improvement in the symptoms of CIDP. Quote from market research: "...wheelchair to walking is the metric that I use more than not, and walking without support is the other one." —Neuro, Hytrulo Non-User <u>V3.0 (September 2025) update:</u> - Renewing DOF packet for continued use of supportive data for the change in INCAT leg score from the initial treatment period baseline (Stage A baseline) to the start of the randomized-withdrawal phase (Stage B baseline). - Note that this analysis differs slightly from the published INCAT leg data in the Stettner et al EAN 2025 poster, which uses the run-in baseline to Stage A last assessment. The Marketing team is requesting renewal of the DOF packet for alignment with the PI and other analyses in the CIVA (which use the Stage A baseline).
Use(s): Please describe the manner in which the following data will be utilized (e.g. multiple assets, multiple agencies, etc.): These data may be used in the following example commercial materials, including but not limited to: - Pre-approval information exchange (PIE) and payer value proposition (PVP) decks - Commercial interactive visual aid (CIVA) - Frequently asked questions (FAQs) - HCP marketing materials - Patient marketing materials - Speaker decks

Content: As shown in Table 14.5.4.6 below: - This post-hoc analysis included 219 patients with a recorded INCAT leg score at the initial treatment period baseline (Stage A baseline) and at least 1 post-baseline INCAT leg score at the randomized withdrawal period (Stage B). - There were 22 patients with a score of ≥ 4 (usually used a wheelchair) before treatment. Of these patients, 59% (13/22) had a score of ≤ 3 (no longer needed to use a wheelchair) after the initial treatment period (Stage A). - There were 119 patients with a score of 2-3 (usually used support to walk) before treatment. Of these patients, 39% (46/119) had a score of ≤ 1 (were able to walk independently) after the initial treatment period (Stage A).
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Table 14.5.4.6: Cross-tabulation of INCAT leg disability score from Stage A Baseline at Stage B Baseline

ANALYSIS SET: mITT

Treatment Group = OVERALL
Domain = INCAT Leg Grade Score

Analysis Visit	INCAT leg score at analysis visit	INCAT leg scores at Stage A Baseline, n(%)				Total N=219
		0-1 N=78	2 N=81	3 N=38	4-5 N=22	
Stage B Baseline	0-1	77 (98.7)	40 (49.4)	6 (15.8)	5 (22.7)	128 (58.4)
	2	1 (1.3)	41 (50.6)	13 (34.2)	4 (18.2)	59 (26.9)
	3	0 (0.0)	0 (0.0)	19 (50.0)	4 (18.2)	23 (10.5)
	4-5	0 (0.0)	0 (0.0)	0 (0.0)	9 (40.9)	9 (4.1)

The denominator is the total number of participants in the mITT analysis set with a Stage A baseline and at least one post-baseline measurement in Stage B and per INCAT leg score at Stage A baseline.

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Please refer to Table 9 below for the information broken down by Stage B treatment group (efgartigimod rHuPH20 or placebo).

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Table 9: Cross-tabulation of INCAT leg disability score from Stage A baseline at last assessment and at best-improvement - Stage B

ANALYSIS SET: mITT

Treatment Group = EFG PH20 SC
Domain = INCAT Leg Grade Score

Analysis Visit	INCAT leg score at analysis visit	INCAT leg scores at Stage A Baseline, n(%)				Total N=110
		0-1 N=41	2 N=42	3 N=19	4-5 N=8	
Stage B Baseline	0-1	41 (100.0)	23 (54.8)	3 (15.8)	3 (37.5)	70 (63.6)
	2	0 (0.0)	19 (45.2)	5 (26.3)	1 (12.5)	25 (22.7)
	3	0 (0.0)	0 (0.0)	11 (57.9)	1 (12.5)	12 (10.9)
	4-5	0 (0.0)	0 (0.0)	0 (0.0)	3 (37.5)	3 (2.7)
Stage B Last Assessment	0-1	41 (100.0)	22 (52.4)	2 (10.5)	3 (37.5)	68 (61.8)
	2	0 (0.0)	18 (42.9)	5 (26.3)	1 (12.5)	24 (21.8)
	3	0 (0.0)	2 (4.8)	11 (57.9)	0 (0.0)	13 (11.8)
	4-5	0 (0.0)	0 (0.0)	1 (5.3)	4 (50.0)	5 (4.5)
Stage B Best-Improvement	0-1	41 (100.0)	26 (61.9)	3 (15.8)	3 (37.5)	73 (66.4)
	2	0 (0.0)	16 (38.1)	8 (42.1)	1 (12.5)	25 (22.7)
	3	0 (0.0)	0 (0.0)	8 (42.1)	0 (0.0)	8 (7.3)
	4-5	0 (0.0)	0 (0.0)	0 (0.0)	4 (50.0)	4 (3.6)

The denominator is the total number of participants in the mITT analysis set with a Stage A baseline and at least one post-baseline measurement in Stage B per treatment and per INCAT leg score at Stage A baseline.

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Table 9: Cross-tabulation of INCAT leg disability score from Stage A baseline at last assessment and at best-improvement - Stage B

ANALYSIS SET: MITT

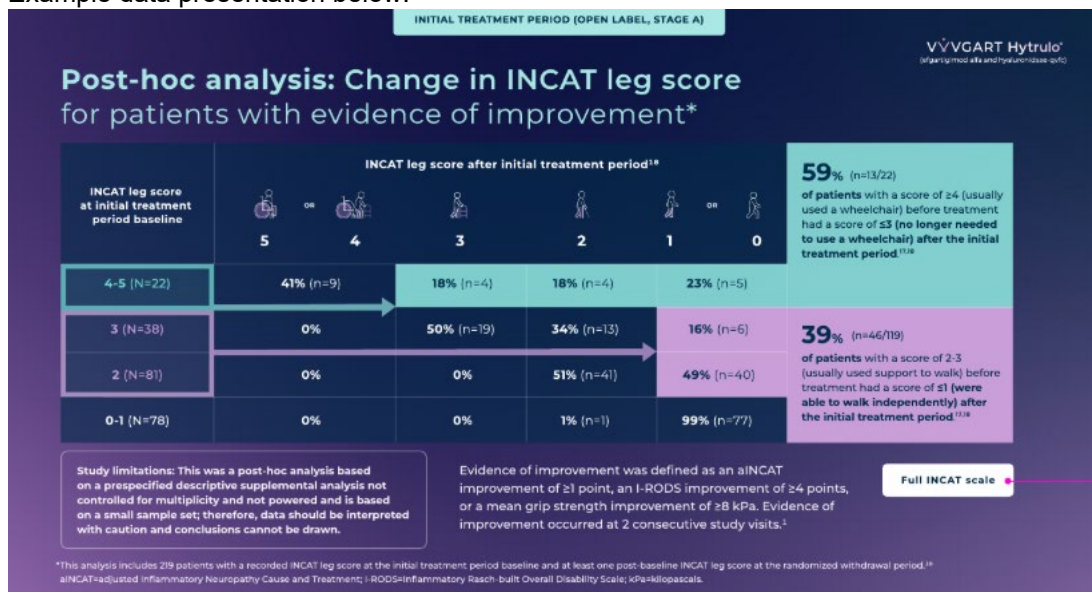
Treatment Group = PBO PH20 SC
Domain = INCAT Leg Grade Score

Analysis Visit	INCAT leg score at analysis visit	INCAT leg scores at Stage A Baseline, n(%)				Total N=109
		0-1 N=37	2 N=39	3 N=19	4-5 N=14	
Stage B Baseline	0-1	36 (97.3)	17 (43.6)	3 (15.8)	2 (14.3)	58 (53.2)
	2	1 (2.7)	22 (56.4)	8 (42.1)	3 (21.4)	34 (31.2)
	3	0 (0.0)	0 (0.0)	8 (42.1)	3 (21.4)	11 (10.1)
	4-5	0 (0.0)	0 (0.0)	0 (0.0)	6 (42.9)	6 (5.5)
Stage B Last Assessment	0-1	29 (78.4)	15 (38.5)	5 (26.3)	3 (21.4)	52 (47.7)
	2	7 (18.9)	17 (43.6)	3 (15.8)	1 (7.1)	28 (25.7)
	3	1 (2.7)	6 (15.4)	9 (47.4)	1 (7.1)	17 (15.6)
	4-5	0 (0.0)	1 (2.6)	2 (10.5)	9 (64.3)	12 (11.0)
Stage B Best-Improvement	0-1	35 (94.6)	22 (56.4)	6 (31.6)	4 (28.6)	67 (61.5)
	2	2 (5.4)	16 (41.0)	4 (21.1)	1 (7.1)	23 (21.1)
	3	0 (0.0)	1 (2.6)	8 (42.1)	1 (7.1)	10 (9.2)
	4-5	0 (0.0)	0 (0.0)	1 (5.3)	8 (57.1)	9 (8.3)

The denominator is the total number of participants in the MITT analysis set with a Stage A baseline and at least one post-baseline measurement in Stage B per treatment and per INCAT leg score at Stage A baseline.

Program: /clinical/argx-113/cidp/argx-113-1802/biostat/staging/ma2023/programs/t_eff_score_shift.sas, Version: 0.7

Example data presentation below:



Note to content creators: please refer to the following published source if data are needed to support the change in INCAT leg score from Run-in Baseline to Stage A Last Assessment:

- Stettner M, Rinaldi S, Allen JA, et al. Effect of efgartigimod PH20 on lower limb function in chronic inflammatory demyelinating polyradiculoneuropathy in ADHERE. Poster at: European Academy of Neurology (EAN); June 21-24, 2025; Helsinki, Finland.

Reference:

ARGX-113-1802 Post-hoc Analysis, 29 May 2024

- Table 14.5.4.6 Cross-tabulation of INCAT leg disability score from Stage A Baseline at Stage B Baseline

ARGX-113-1802 Supplemental Analysis, 06 Feb 2024

- Table 9 Cross-tabulation of INCAT leg disability score from Stage A Baseline at last assessment and at best-improvement – Stage B

Talking Points:

- A total of 322 patients received efgartigimod rHuPH20 SC in ADHERE Stage A. This post-hoc analysis shows the change in INCAT leg score for patients with evidence of clinical improvement*. The analysis included 219 patients with a recorded INCAT leg score at the initial treatment period baseline (Stage A baseline) and at least 1 post-baseline INCAT leg score at the randomized withdrawal period (Stage B).
 - *Evidence of improvement in the open-label Stage A was defined as an aINCAT improvement (decrease) of ≥ 1 point, an I-RODS improvement (increase) of ≥ 4 points, or a mean grip strength improvement (increase) of ≥ 8 kPa, for 2 consecutive visits.
- 59% of patients (13/22) with a score of ≥ 4 (usually used a wheelchair) before treatment had a score of ≤ 3 (no longer needed to use a wheelchair) after the initial treatment period.
- 39% of patients (46/119) with a score of 2-3 (usually used support to walk) before treatment had a score of ≤ 1 (were able to walk independently) after the initial treatment period.

Checklist for additional requirements to appropriately describe analyses:

X Type of analysis Prespecified analysis

X Analysis set mITT Stage B

☐ Mean at baseline

☐ Confounders, if applicable

X Limitations of analysis This was a post-hoc analysis based on a prespecified descriptive supplemental analysis not controlled for multiplicity and not powered and is based on a small sample set; therefore, data should be interpreted with caution and conclusions cannot be drawn.

Approval

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EMA Medical Leadership	Name: Roland Derwand <i>Roland Derwand</i>
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